

THE NEWBORN MANUAL

A practical guide for new parents — Text-Only Edition

Congratulations on Your New Baby!

As your child's pediatrics office, we would like to share some information that we believe will be helpful when caring for your newborn. The good news is that, in general, common sense is the most important factor in infant care.

Remember that every baby is an individual with his or her own personality, and each will react differently to the same situation. Try not to compare babies or their reactions, as there are wide ranges of standards for weight, growth and development for infants, just as there are for adults. This is why we can only offer general thoughts and advice in this book.

Try not to worry about small problems or let well-meaning loved ones concern you with their advice. The best way to safeguard your baby's health is to have regular examinations in our office.

Throughout the first year, we'll be here with detailed instructions on how to care for your baby. Please reach out to our office whenever you have a question or need advice. Sometimes, we can provide a definitive answer to a puzzling problem. Other times, just talking about the problem will help you to come to your own solution.

We want you to depend on us as child specialists. Together, we can help you develop your own parenting style.

— *Your UCLA Health pediatrics team*

Chapter 1: About Your Baby

Newborn Behavior: What Do I Do if My Infant is...?

Colicky: The definition of colic is crying that occurs for more than three hours per day, more than three days per week and lasts for more than three weeks. It affects at least 20 percent of all babies. Infants with colic can have multiple episodes of crying, fussing and irritability that may develop into agonizing screaming. Symptoms of colic usually occur in the evenings and begin in the second week of life. Colic typically peaks at six weeks and resolves by four months. While there are few medical interventions that are helpful for colic, feel free to ask your pediatrician for suggestions.

Crying: Crying is normal in all infants for up to three hours a day. Babies cry when they are hungry, need their diaper changed, are too hot or too cold, are sleepy, or need to be burped. If you cannot determine the cause of crying, check your baby's temperature and make sure he or she does not have a fever. If he is still crying despite checking all of the above, try soothing your baby by swaddling him tightly in a blanket, rocking him, and singing or "shushing" him. You can also try using a pacifier.

Some things newborn babies commonly do may concern parents, but they are not signs of illness. Most are harmless reflexes caused by an immature nervous system that disappear in two to three months, including:

- Chin trembling or lower lip quivering
- Frequent yawning and hiccups
- Passing gas
- Periodic breathing: rapid non-labored breathing followed by a brief pause (less than 10 seconds) and then normal breathing
- Noises caused by breathing or movement during sleep, sneezing, and throat clearing or gurgling sounds caused by secretions in the throat
- Spitting up (small amounts) or burping
- Startle reflex (Moro reflex): a brief stiffening of the body with arms in an "embracing" position in response to noise or movement
- Straining with bowel movements

Trembling or Jitteriness: Trembling or jitteriness of arms and legs during crying is common. While jitters are common in infants, convulsions are rare. During convulsions, babies may make jerking movements, blink their eyes and suck rhythmically with their mouths while not crying. If your baby is trembling but isn't crying, give him or her something to suck on. If the trembling doesn't stop during sucking, call your doctor's office immediately.

Temperature Concerns: A newborn's temperature is normally higher than those of older children, averaging approximately 99.5 degrees Fahrenheit (37.5 degrees Celsius) during the first six months of life. The most accurate way to take a newborn's temperature is to use a digital rectal thermometer. If a newborn's temperature is over 100.4°F (38.0°C), you must call your doctor's office immediately.

Chapter 2: Newborn Bodies

Heads

- **Fontanel:** The fontanel is the diamond-shaped soft spot found on the top front part of the newborn skull. It is covered by a thick, fibrous layer of tissue and is safe to touch. Its purpose is to allow rapid growth of the brain. It normally closes over with bone when your baby is between 9 and 24 months old.
- **Molding of the Head:** Molding refers to the long, narrow, cone-shaped head that results from passage through a tight birth canal. This compression can temporarily hide the fontanel. The head should return to a normal shape in a few days.

- **Caput:** Swelling on top of the newborn head or throughout the scalp that occurs when fluid is squeezed into the scalp during birth. It typically clears within a few days.
- **Cephalohematoma:** A lump on the head that occurs when blood collects under the skin. It is caused by friction between the infant's skull and the mother's pelvic bones during birth, typically appearing on the second day of life. It should disappear completely by 2 to 3 months of age.

Eyes, Mouth & Feet

- **Eye Color & Hemorrhage:** Eye color is often uncertain until 6 months. Some babies have a harmless, flame-shaped hemorrhage on the side of the eye caused by broken surface blood vessels during birth, which reabsorbs within 2 to 3 weeks. Puffy eyelids from delivery pressure usually resolve within a week. Continuous watering may indicate a blocked tear duct.
- **Sucking Callus or Blister:** Occurs in the center of the upper lip from constant friction during bottle or breastfeeding and disappears when cup feedings begin.
- **Teeth:** A tooth at birth is rare (10% extra teeth without roots, 90% prematurely erupted normal teeth). A dentist should evaluate them as loose teeth pose a choking hazard.
- **Tongue-Tie:** A short, tight band on the underside of the tongue that connects to the floor of the mouth. It usually stretches with time, but let your provider know if it impacts nursing.
- **Epithelial Pearls:** Little white cysts containing clear fluid along the gum line or roof of the mouth that disappear after 1 to 2 months.
- **Foot Direction:** Feet may turn in any direction due to cramped quarters in the womb. As long as they are flexible and easily moved to a normal position, they will naturally straighten between 1 and 6 months of age.

Legs, Torso & Genitals

- **Tibial Torsion & Bowed Legs:** The lower leg bone (tibia) normally curves inward in newborns. Legs may look bowed and feet pigeon-toed; this naturally straightens 6 to 12 months after the child begins walking.
- **Swollen Breasts:** Many male and female babies develop swollen breasts due to maternal hormones crossing the placenta. This persists for 4 to 6 months. Never squeeze the breast.
- **Body Hair (Lanugo):** Fine downy hair present on the back and shoulders that rubs off with normal friction by 2 to 4 weeks of age.
- **Male Genitals:** The scrotum may be filled with clear fluid (hydrocele), which can take 6 to 12 months to clear completely. Do not retract a tight foreskin; it separates naturally by 5 to 10 years of age. Erections are normal reflexes, often triggered by a full bladder.
- **Female Genitals:** The labia minora and hymen may be swollen from maternal hormones. Small pink tissue projections (hymenal tags) shrink within 2 to 4 weeks. Clear, white, or pink/blood-tinged discharge (false menstruation) during the first week is normal.

Chapter 3: Feeding Your Baby

Feeding Basics & Responsive Feeding

Exclusive breastfeeding is recommended for the first six months, followed by continued breastfeeding as complementary foods are introduced up to two years and beyond. All newborns need to wake and feed frequently—at least 8 or more times in 24 hours. Wake newborns every three hours at night to feed at first. Once your baby is gaining weight well (usually by the end of week two), you can let them sleep and wake you on demand.

Feeding Cues: Responsive feeding reduces underfeeding and overfeeding. Look for early cues (stirring, mouth opening, turning head, rooting, hand to mouth) rather than waiting for late cues (crying, lots of movement, turning red). Calm a crying baby before feeding using skin-to-skin contact, cuddling, or talking.

Breastfeeding & Formula-Feeding Guidelines

- **Colostrum & Transitional Milk:** For the first few days, breasts produce colostrum—a thick, yellow-orange fluid rich in protein and antibodies. Mature milk "comes in" between days 2 and 6, transitioning into milky-white milk. Frequent feedings (every 2 to 3 hours) help build supply and prevent severe engorgement.
- **Formula Basics:** Use an FDA-approved iron-fortified infant formula. Standard types include milk-based, soy-based, and simple protein formula. Never feed low-iron formulas or other milks (cow's, goat's, almond, soy drinks) during the first year.
- **Safe Formula Preparation:** Always wash hands and sterilize all bottles and nipples in boiling water for two minutes. Bring safe tap water to a boil for two minutes, let it cool for no more than 30 minutes, then measure the water into the bottle **before** adding formula powder to ensure the correct ratio. Store prepared formula in the refrigerator for up to 24 hours. Warmed formula must be used within an hour. Discard any formula remaining in a bottle one hour after a feeding starts.
- **Bottle Feeding Safety:** Cradle your baby in an upright, semi-reclined position. Never feed babies flat on their backs or prop the bottle, as this increases the risk of choking and ear infections. Never put your baby to bed with a bottle.

Chapter 4: Pee & Poop

By days 4–7, your baby will have roughly 5 to 10 bowel movements and 5 to 6 wet diapers every 24 hours. Track these numbers closely during the first week.

Uric Acid Crystals ("Red Dust"): Parents may see red or salmon-colored dust in the diaper on Day 1, which is often mistaken for blood. This is actually uric acid crystals caused by concentrated urine and is normal in the first days.

Stool Consistency and Color Progression:

- **Day 1 (Meconium):** Black, tarry, sticky poop.

- **Day 2–3 (Transitional Stool Stage 1 & 2):** Greenish-black to greenish-yellow, less sticky, more watery consistency.
- **Day 4–5 (Transitional Stool Stage 3):** Mostly mustard yellow with some green, slightly chunky.
- **Day 7 onward (Normal Newborn Stool):** Mustard yellow, clumpy, chunky, or watery (liquid and seedy for breastfed babies; dark yellow and soft for formula-fed babies).

Note: If stool is ever very hard, or looks entirely white, red, or bloody, call your doctor immediately.

Chapter 5: Bathing & Body Care

Bathing: Sponge bathe your baby until his or her umbilical cord stump heals and drops off, which can take up to three weeks. Keep the cord stump dry. Once fully healed, move to tub or sink baths. Wash the face and eyes (from inside corner outward) with plain water first, then the body with mild soap, and finish with baby shampoo so the head isn't wet for too long. Dry thoroughly, paying special attention to skin creases and folds to prevent chafing.

Circumcision Care: At each diaper change, gently pull the skin back and place a piece of gauze with pure petroleum jelly (without perfumes) on top of the site to keep it from sticking to the diaper. If a Plastibell plastic ring was used, it takes 5 to 7 days to fall off naturally; do not pull on it. Call your doctor if you notice persistent redness, foul-smelling discharge, sudden swelling, or more than a drop or two of blood.

General Grooming: Clean the nose openings gently with damp cotton or a Q-tip (never insert sharp/hard objects into the nose or ears). File fingernails in a slightly rounded shape and toenails straight across to prevent ingrown nails.

Chapter 6: Home & Travel Safety

- **The Newborn's Room:** Keep the room temperature around 68 to 72°F (20 to 22.2°C) for the first few weeks. Ensure good indirect ventilation. Never use an open gas heater in a baby's room.
- **Safe Sleep Environment:** The American Academy of Pediatrics recommends that infants sleep in the same room as their parents (but on a separate surface) until at least 6 months old. Always place babies to sleep on their backs on a firm mattress with a snugly fitted sheet. Never place blankets, bumpers, pillows, or stuffed toys in the crib. Avoid letting your baby sleep on a couch, armchair, stroller, swing, or reclining sleeper.
- **Car Safety:** Infants and toddlers must always ride in a tightly installed, rear-facing car safety seat until they are at least 2 years old. Ensure the harness straps are in slots at or below the shoulders and the chest clip is at mid-chest level. The seat should be at the correct angle so the infant's head does not flop forward.
- **Air Travel:** Air travel is generally safe for healthy infants starting around 1 month of age. Nurse or bottle-feed during takeoff and landing to relieve ear pressure. The safest place for an infant on an airplane is in a government-approved Child Restraint System (CRS) safety seat (under 16 inches wide) secured in a window seat.

Chapter 7: For Mothers

Postpartum Depression (PPD): Perinatal mood and anxiety disorders affect 10 to 15 percent of women (1 in 7 moms). Unlike the "baby blues," which resolve on their own within two weeks, postpartum depression does not go away without professional treatment. Symptoms include ongoing anxiety, crying a lot, trouble bonding with the baby, intense guilt, or helplessness. Perinatal anxiety may cause a racing heart, rapid breathing, and obsessive thoughts. If you experience any of these signs, contact your healthcare provider immediately. Counseling and medication are highly effective.

Sleep & Nutrition: Sleep when the baby sleeps and let family or partners handle household tasks. Eat a healthy, balanced diet to aid healing and drink plenty of fluids, especially if breastfeeding. Check the public NIH *LactMed* database to verify the safety of any specific medications while breastfeeding.